

KISS: Contraception in the Perimenopause [BJGP 2015;65:e630](#)

1. Combined hormonal contraception (CHC) - an effective option that should provide the same symptomatic benefit as HRT but we need to be aware of the UKMEC criteria

- Age 40 or over is UKMEC 2, so in the absence of other risk factors remains an option for older women, although prescription >50 years not advised by the paper
- Common C/Is include smokers >35y, hypertension, migraine with aura, complicated diabetes

2. Mirena plus oestrogen-only HRT - gold standard option

- Mirena is licensed as contraception and as the progestogen component of HRT
- 65% will achieve amenorrhoea by 1 year so offers a potentially bleed-free option
- If fitted for contraception or the progestogen element of HRT when <45yo the IUS should be changed at 5 years; if fitted purely for heavy bleeding it can be left as long as it proves effective

3. HRT plus progestogen-only contraception (POC)

- The FSRH currently recommend that combined HRT is still required despite POC use; however, some experts argue that progestogen pills or implant can be used in the same way as the Mirena is to provide endometrial protection.

KISS: Mental Health in the Peri-Menopause [NICE NG23](#) [BMJ Best Practice 2020](#)

Background:

- Mood changes and mental health disorders are common in the peri-menopause and early post-menopausal periods.
- However, there is limited evidence and a lack of consensus or recommendations as to how to assess and manage mental health problems during the peri-menopause.
- Menopause symptoms can complicate, co-occur and overlap with depressive symptoms, making assessment difficult.
- Most women who experience a major depressive episode during the peri-menopause have experienced a prior episode of depression, but there is obviously a huge spectrum of mental health illness that can present in the peri-menopause.

Assessment and Management:

- Assess for previous depressive episodes.
- Assess for suicidal ideation in women with severe depressive symptoms, as usual.
- Menopause-specific mood disorder scales/questionnaires do not exist, but other known screening tools (e.g. PHQ9) can be considered
- **NICE recommendations:**
 - Consider HRT to alleviate low mood that arises as a result of the menopause.
 - Consider CBT to alleviate low mood or anxiety that arises as a result of the menopause.
 - Ensure that menopausal women and healthcare professionals involved in their care understand that there is no clear evidence for SSRIs or SNRIs to ease low mood in menopausal women who have not been diagnosed with depression.
- **BMJ Best Practice:**
 - Short-term use of HRT may improve mood and depressive symptoms during the menopausal transition and in early menopause but study results are inconsistent.
 - Mood disorders often improve with HRT, although conventional antidepressants may be more effective in depression.
 - There is a lack of good-quality evidence relating to CBT, but given the low risk of harms and strong evidence in the general population, it is a reasonable option.

KISS: Premature Ovarian Insufficiency [NG23](#)

Diagnose premature ovarian insufficiency (POI):

- In the presence of menopausal symptoms, including absent or infrequent periods, AND
- Elevated FSH levels (>30mIU/ml) on 2 blood samples 4-6 weeks apart.

Treatment

- *Offer either COCP or HRT* unless contraindicated
- Treatment should be continued until at least the expected age of natural menopause (average age 52)
 - No evidence shows one is more effective than the other
 - HRT may have a beneficial effect on blood pressure compared to COCP
- Consider referral to a health professional with experience in POI
- If hormonal therapy is contraindicated, advise about bone and cardiovascular health and symptoms management

NB Learning Points & Quality Improvement Ideas: Menopause

- Menopausal symptoms can last for up to 12 years and significantly impair quality of life
- HRT is an effective treatment with the transdermal route safer than oral HRT
- Micronised progesterone appears to have lower risks than other progesterone types, and in combination with transdermal oestrogen may represent the optimal HRT regime
- Audit patients using HRT. What percentage of your population use transdermal HRT? Could more women have the potential benefits of transdermal HRT discussed and see if this alters their choices.
- Patient resources:
 - [Women's Health Concern](#) - patient-facing arm of the BMS
 - [Rock My Menopause](#) - patient-facing campaign of the Primary Care Women's Health Forum
 - [Daisy Network](#) - information and assistance of premature ovarian insufficiency

URINARY INCONTINENCE

Menopause has had a very high media profile recently, but it isn't the only condition affecting women which has not had the recognition deserved.

Both stress incontinence and overactive bladder can cause significant morbidity impacting the daily life of women of all ages. These conditions are extraordinarily common and as yet are rarely discussed with medical professionals. Two new articles from the BJGP ([2022;72\(717\):185](#)) and BMJ ([2021;375:e063526](#)) bring new insights to these problems.

1 **Overactive Bladder** [BMJ 2021;375:e063526](#)

While the management of overactive bladder has been well documented in national guidance ([NICE NG123 2019](#) - see KISS) this recent review in the BMJ provides clinicians with some practical tips for patients.

Prevalence of overactive bladder syndrome (OBS) is suggested at 20%, more commonly in women and increases with age. It is discussed infrequently (compared to its true prevalence) due to embarrassment or acceptance as a part of ageing.

There may be no clear cause but known drivers include excessive or inadequate intake (the latter due to concentrated urine irritating bladder mucosa), and carbonated or caffeinated drinks. The 'latch key' effect is also well documented: people needing to urinate as soon as they get home and put the key in the door; a similar effect is seen in a range of scenarios such as proximity to a toilet.

The paper highlights a range of differentials that should be considered and features that point away from OBS:

KISS: Differential Diagnosis of Overactive Bladder Syndrome [BMJ 2021;375:e063526](#)

Small & variable volume voids, normal fluid intake	Probable OBS
Small & fixed volume voids, normal fluid intake	Possible interstitial cystitis
Small volume voids, low fluid intake	Restrictive behaviour to control symptoms
Large volume voids	Usually large intake, consider DM, diabetes insipidus, hypercalcaemia, diuretics
Nocturnal polyuria	Possible CHF, also with ageing, sleep apnoea, also behavioural if restricting intake during day and increased in evenings (seen with certain jobs, e.g. taxi drivers, medics...)
Dysuria, haematuria, suprapubic pain	Not seen in OBS, consider UTI, STI, bladder Ca, interstitial cystitis
LUTS for only weeks > months	Consider bladder Ca, by the stage it causes Sx haematuria (visible or non-visible) is very likely to be present
Hesitancy, poor flow, incomplete bladder emptying	Possible reduced bladder contractility, can result in large residual volume which drives OBS symptoms
Dragging sensation, bulging	Possible prolapse or fibroids
Neurological Hx (e.g. CVA, Parkinson's, MS)	Possible neurogenic bladder

It is important to examine women to exclude a palpable bladder, abdominal mass, vaginal atrophy and pelvic prolapse. Urinalysis should be performed to exclude haematuria and infection.

► *Managing overactive bladder syndrome*

Given the significant limitations the condition can cause for people, [good patient information and support groups](#) are recommended. The authors also highlight the ['Just Can't Wait' card from the Bladder & Bowel Community](#) which shows businesses that this person has a medical reason to need the toilet quickly.

Conservative measures can be effective and durable - bladder training (min. 6/52), weight loss if obesity, stopping carbonated & caffeinated drinks, optimising fluid intake (~2-2.5L/day). Patients with mixed OBS and stress urinary incontinence can benefit from pelvic floor muscle training.

Vaginal atrophy can be an important barrier to improvement and in this group, vaginal oestrogen is indicated.

Pharmacological therapy may be tried where conservative measures have failed:

- **Anticholinergics are the 1st line treatment** but limited by their side effects - only 8-25% continue the treatment by 12 months. Selective anticholinergics such as solifenacin may cause fewer side effects.
 - They should be avoided in the elderly, frail and those with a high anticholinergic burden.
- Mirabegron, a β_3 -adrenoceptor agonist, is 2nd line in NICE guidance but may be a better choice in these groups if drug therapy is still desired.
 - Caution is required in hypertension, renal and liver impairment.
- A combination of mirabegron & solifenacin is supported by trial data but is not currently part of national guidance.

Secondary care options include further investigation to confirm the diagnosis, botox injections, sacral neuromodulation and, where these fail, percutaneous posterior tibial nerve stimulation.

2 **Stress Urinary Incontinence** [BJGP 2022;72\(717\):185](#)

As the authors point out, given stress urinary incontinence (SUI) affects around a third of women it is surprising that it "does not grab more attention". There appears an acceptance that this is a condition that 'just happens' to some women and is 'something to be managed', an attitude further entrenched by the normalisation of incontinence pads in retail stores. While an easy way to self-manage health issues is welcome, it should not come at the expense of education on genuine treatment options.

► *What are the causes of SUI?*

These are multifactorial and there may be multiple defects before SUI develops due to continence control consisting of several mechanisms. Childbirth, repetitive insults such as chronic coughing/constipation/etc., and high BMI may contribute. SUI is also common during pregnancy. SUI commonly affects two groups: around 20% of women aged 18-30, and aged 70+.

Prevention is achievable through pelvic floor muscle training (PFMT) and has shown to improve labour and delivery outcomes, and sexual function when undertaken during late pregnancy and postpartum, and could prevent 25% of SUI in primips. Data shows PFMT is also effective for established SUI. The paper argues there are many opportunities to discuss possible SUI with women and offer preventative education, such as ante- and post-natal visits.

► *Pelvic floor muscle training*

The key treatment is PFMT. NICE advises that women should undertake these exercises throughout life.

However, as the paper explained "research has found that women find it difficult to understand and perform PFMT", but that some basic, verbal guidance can help the majority.

Initially, women will need to undertake strengthening exercises and then they can move to maintenance. Regular PFMT can result in automatic contraction of the pelvic floor during activities when leakage may occur. It is also possible to use a pre-emptive 'squeeze and lift' before & during triggers of SUI, e.g. lifting, sneezing.

KISS: Practical Tips on Pelvic Floor Muscle Training [BJGP 2022](#)

- **Educate on correct pelvic floor muscle contraction**
 - To exercise these muscles squeeze your vagina then lift up inside
 - Hold the squeeze for as long as possible, counting the seconds, then release
 - Avoid breath-holding or squeezing the buttocks
 - The paper advises against stopping urine mid-flow as an exercise
- **Recommended regime**
 - Find how many squeeze, lift and holds the patient can do, and for how long
 - Aim for 2 sets of 8 maximal contractions with 8 seconds hold, 2-3x daily
 - Strengthening (and symptom improvement) may take 12-16 weeks
 - Maintenance: 2 sets of 8 maximal contractions with 8s hold, daily or 3x/wk min.
- **Counter-balance contraction (the Knack technique)**
 - Pre-emptive squeeze and lift before and during activities which may precipitate leakage

[Useful instructional video](#) from NHS Oxford University Hospitals is available free on youtube - the regime recommended there is very slightly different

► *What else can help?*

NICE suggests weight loss may be beneficial for women with overweight or obesity. If after 12 weeks of PFMT there is no improvement, referral to a community physio or continence service is recommended. This can then inform onward referral to specialist care if there is still failure to improve - NICE indicates further options include duloxetine and surgery.

NB Learning Points: Urinary Incontinence

- Avoidance of triggers and bladder training are the cornerstone of overactive bladder syndrome management, and pelvic floor muscle exercises for stress urinary incontinence. Weight loss in patients with obesity can also prove beneficial.
- Anticholinergics are 1st line drug therapy for OBS but limited due to side effects and should be avoided in the elderly, frail and those already with a high cholinergic burden. Mirabegron is a suitable 1st line alternative.
- Use of mesh procedures is currently on hold for women with SUI and POP, however, there are a range of effective non-surgical interventions that may be offered
- Women with surgery involving mesh and possible complications should be referred to a specialist centre for further assessment. The list is available [here](#).

NB Quality Improvement Ideas: Urinary Incontinence

- Perform a case review of your patients on anticholinergic medication. Are they receiving other medications with a high anticholinergic burden (e.g. TCAs, antihistamines)? Have anticholinergic side effects/symptoms been discussed with the patient? Should the anticholinergic be stopped?
 - Consider with hx of memory impairment, disorientation, instability, change in vision, etc.

KISS: Urinary Incontinence and Pelvic Organ Prolapse in Women: Management [NG123 2019](#)**Assessment**

- **Categorise the cause of urinary incontinence**
 - ***Stress, mixed, urgency/overactive bladder***
- Examination: include PV assessment of pelvic floor muscle contraction
- Urine dipstick +/- MC&S if positive leucocytes/nit
- Undertake a bladder diary for ≥ 3 days as part of the initial assessment
- Refer for bladder scan if possible voiding dysfunction or recurrent UTI
- Do not undertake urodynamic testing with SUI or stress-predominant mixed UI
- Imaging and cystoscopy are not routinely recommended

Referral

- Consider referral if persisting pain, palpable bladder, pelvic mass, associated faecal incontinence, suspected neurological disease, voiding difficulty, fistulae, previous continence surgery, previous pelvic cancer surgery or radiotherapy
- Refer via 2ww for women with haematuria or recurrent or persistent unexplained UTI

Non-surgical management of Stress UI or Mixed UI

- ***Offer a trial of supervised pelvic floor muscle training of at least 3 months as 1st line***
 - Involves at least 8 contractions TDS
 - Continue if beneficial
 - Electrical stimulation/biofeedback should not be used routinely but may aid those unable to contract their pelvic muscles themselves
- Do not offer intravaginal or intraurethral devices to prevent leakage for routine management
- Duloxetine is a possible second-line option for women who wish to avoid surgery

Non-surgical management of Urgency IU or Mixed UI

- ***Lifestyle measures***
 - Recommend a trial of caffeine reduction, fluid intake modification, weight loss if BMI >30
 - Offer bladder training for ≥ 6 weeks as 1st line
 - If not satisfactory benefit consider medication + bladder training
 - Do not offer transcutaneous sacral/posterior tibial nerve stimulation
 - Percutaneous post. tibial nerve stimulation may be used if continuing Sx and not using Botox
- ***Medication***
 - ***Offer intravaginal oestrogen in postmenopausal women with vaginal atrophy.*** Do not offer systemic HRT

KISS: Urinary Incontinence and Pelvic Organ Prolapse in Women: Management [NG123 2019](#)

Non-surgical management of Urgency IU or Mixed UI

■ **Medication - anti-cholinergics**

- Before starting, discuss the likelihood of benefit, common s/e, need for $\geq 4/52$ trial and uncertain long-term effects on cognition. Consider risks and cautions/contra-indications, e.g. dementia, poor bladder emptying, overall anticholinergic burden
 - Do not offer imipramine, propantheline or flavoxate
 - Do not offer oxybutynin immediate release to older women at risk of worsening health
- **Offer anticholinergic with a low acquisition cost**
 - If 1st medication is not well tolerated, offer an alternative with a low acquisition cost
 - Offer transdermal preparation if unable to use oral
 - **Mirabegron is an option** where antimuscarinic drugs are not tolerated/contraindicated or clinically ineffective ([TA290](#))
- **Desmopressin may be considered specifically to reduce nocturia.** Caution in women with CF, or $>65y$ + CVD or hypertension

■ **Invasive Procedures**

- Urodynamic testing will be required to demonstrate detrusor overactivity. If present:
- Botox injection may be considered. Women need to be willing to undertake intermittent catheterisation as this is a possible adverse effect. Benefits should last 6 months+.
- Percutaneous sacral nerve stimulation
- Severe and refractive cases may be offered augmentation cystoplasty or urinary diversion

Non-surgical management of Pelvic Organ Prolapse

- Lifestyle measures include losing weight if BMI >30 , minimising heavy lifting + preventing constipation
- Consider vaginal oestrogen if vaginal atrophy, including ring, if cream/pessaries difficult to use
- Consider a 16/52+ programme of supervised pelvic floor muscle training
- Consider a vaginal pessary with symptomatic prolapse. Should be removed at least once every 6/12.

Surgical management of Stress UI

- Current options include colposuspension or autologous rectus fascial sling, with mesh slings currently restricted. Intramural bulking agents are a further alternative.
- NICE have produced a [useful patient decision aid](#).

Surgical management of Pelvic Organ Prolapse

- Again NICE have produced a useful patient decision aid for [uterine](#) and [vaginal vault](#) prolapse.
- Options include vaginal hysterectomy and/or sacrospinous fixation or Manchester repair, mesh is currently off-limits in most areas, except in research settings.